HISTORY OF PRESENT ILLNESS: A 28-year-old male presents for comprehensive evaluation of recurrent paroxysmal neurological events occurring over the past 14 months. The patient describes a stereotyped prodromal sensation (aura) characterized by an ascending epigastric 'rising sensation' accompanied by a sudden feeling of intense déjà vu and an unpleasant olfactory perception of burning rubber lasting 15 to 30 seconds. According to his partner, this is followed by behavioral arrest, blank facial stare, unresponsiveness to vocal commands, and repetitive involuntary semi-purposeful manual automatisms (fumbling with his shirt buttons) and lip-smacking. These episodes last approximately two minutes, followed by a 20-minute period of post-ictal confusion, expressive word-finding difficulty, and intense retrograde amnesia for the event. He has had four such episodes, with the most recent event two days ago secondary to sleep deprivation. He denies generalized convulsions, tongue biting, or urinary incontinence.

NEUROLOGICAL EXAMINATION:

Mental Status: Awake, fully oriented, fluent speech without dysnomia, score 30/30 on Montreal Cognitive Assessment (MoCA). Intact short-term recall (5/5 words at 5 minutes).

Cranial Nerves: Visual fields full to confrontation. Optic fundi sharp without papilledema. Pupils equal and reactive. Cranial nerves II-XII intact.

Motor Examination: Normal muscle bulk and tone throughout all four extremities. Strength 5/5 in all muscle groups.

Reflexes: Deep tendon reflexes 2+ symmetrically throughout; plantar responses flexor.

Coordination: Finger-to-nose and rapid alternating hand movements performed smoothly without dysmetria. Negative Romberg sign. Normal tandem gait.

DIAGNOSTIC STUDIES:

Prolonged Video-Electroencephalogram (vEEG) Monitoring: Demonstrates normal posterior dominant 10 Hz alpha rhythm. During stage II sleep, there are frequent, high-voltage, sharp-and-slow wave discharges localized over the left anterior temporal region (electrodes F7-T3) with phase reversal, confirming regional epileptiform irritability.

Epilepsy-Protocol 3-Tesla Brain MRI: High-resolution coronal T2-weighted and FLAIR sequences demonstrate volume loss of the left hippocampus with hyperintense FLAIR signal and loss of normal internal digitations, consistent with left mesial temporal sclerosis.

ASSESSMENT AND TREATMENT PLAN:

1. Focal epilepsy with focal impaired awareness seizures secondary to left mesial temporal sclerosis.

2. Initiate anti-seizure medication: Start oral levetiracetam 500 mg BID, titrating to 1,000 mg BID over two weeks.

3. Advise strict seizure safety precautions: Driving prohibition per state legal mandate, avoidance of unmonitored swimming, cooking over open flames, or working at heights.

4. Referral to comprehensive epilepsy center for potential future surgical evaluation (selective amygdalohippocampectomy or laser interstitial thermal therapy) if drug resistance develops.